ERLANGER HEALTH | STRATEGY & PLANNING

Service-Line Growth and Community Access Dashboard

A public-data baseline for inpatient and outpatient Medicare demand, market position, quality standing, ZIP-area (ZCTA) access and negotiated pricing across the service area.

- Medicare inpatient & outpatient 2022–2024
- CMS quality & HCAHPS · CDC PLACES by ZCTA
- Price transparency files, as of 30 Sep 2025



What the data says, and what we are asking for

Five findings from the 2024 Medicare, quality and access files — and the three decisions they support

THE FIVE FINDINGS

01 **Payments are flat while share slips.** Inpatient discharge share fell 33.0% to 29.2% since 2022, yet Medicare payments held near $71M — revenue per case and outpatient absorbed the shift.

02 **Orthopedics is the franchise.** 45% inpatient share, $10.1M inpatient plus $5.6M outpatient, and the only line growing in both settings — up 17.7% on 2023.

03 **Cardiology is the recapture.** 559 discharges against Memorial's 1,340. A 10-point share shift is worth roughly $4M in inpatient payments.

04 **Experience, not clinical quality, is the gap.** No mortality measure below national, but nurse communication and cleanliness sit at 2 of 5 stars — the clearest measured gap to Memorial.

05 **Need concentrates in the urban core.** Five ZIP areas (ZCTAs) near the main campus carry the region's heaviest disease burden, poverty to 45% and up to 37% of households with no vehicle.

THE THREE DECISIONS

Fund the cardiology recapture case. Business case and referral plan for the ~780 Medicare discharges now sitting with Memorial.

Commission the orthopedic capacity test. Demand is proven in both settings; confirm whether capacity, not demand, is the constraint.

Authorize a contract review. Reopen the lowest-paying commercial terms in oncology and behavioral health once the volume case is built.

Sources: CMS Medicare Inpatient and Outpatient Hospitals 2022-2024; CMS Care Compare and HCAHPS; CDC PLACES 2024; Erlanger CHNA FY 2024; price transparency files.

What the dashboard shows

Erlanger (CCN 440104) vs. Memorial and Parkridge · Original Medicare fee-for-service, 2024

$71.1M

Total Medicare payments, 2024

$53.2M inpatient, $17.9M outpatient

3,399

Inpatient discharges

Down 12.9% from 2022

4,325

Outpatient services

Flat and stable since 2022

36.9%

Share of market Medicare payments

29.2% of inpatient discharges

- **Outpatient is the stable quarter of the business.** Inpatient discharges fell 13% since 2022 while outpatient held at roughly 4,300 services and $17.9M — a quarter of all Erlanger Medicare payments.
- **Orthopedics leads on both sides of the house.** 45% inpatient market share and the single largest outpatient APC: 359 Level 5 musculoskeletal procedures worth $3.37M.
- **Cardiology is the largest inpatient leak, but not outpatient.** Inpatient cardiac discharges fell 29% to 559 against Memorial's 1,340, even as outpatient electrophysiology and ICD work delivers $1.9M.
- **The sharpest access gap is urban, not rural.** Five ZIP areas (ZCTAs) near the main campus carry the region's heaviest disease burden, poverty to 45%, and up to 37% of households with no vehicle.
- **Negotiated prices vary more than care does.** Under the Chattanooga outpatient facility view, oncology and behavioral-health rates each span roughly 5-fold across 44 payers. BCBST pays roughly twice Humana on cardiology.

Sources: CMS Medicare Inpatient and Outpatient Hospitals by Provider and Service (2022–2024); CMS Hospital General Information; CMS HCAHPS; CDC PLACES 2024 by ZCTA; U.S. Census ACS 2020–2024; Erlanger 2025 CHNA; hospital price transparency files as of 30 Sep 2025.

01

Where the money is

Inpatient DRGs, outpatient APCs and how volume and payments have moved across the Chattanooga market

Where the inpatient money is: top DRGs, 2024

Erlanger — estimated Medicare payments = discharges × average Medicare payment per discharge

DRG	Description	Service line	Discharges	Avg Medicare pmt	Est. Medicare pmt
871	Septicemia or severe sepsis, with MCC	Infectious disease	306	$14,517	**$4.44M**
003	ECMO or tracheostomy with ventilation >96 hrs	Surgery & trauma	20	$137,151	**$2.74M**
853	Infectious disease with O.R. procedure, with MCC	Infectious disease	69	$37,462	**$2.58M**
470	Major hip and knee joint replacement	Orthopedics & spine	138	$13,947	**$1.92M**
023	Craniotomy with major device implant	Neurosurgery	41	$41,315	**$1.69M**
025	Craniotomy and endovascular intracranial procedures	Cardiology & vascular	45	$32,927	**$1.48M**
291	Heart failure and shock with MCC	Cardiology & vascular	133	$9,933	**$1.32M**
329	Major bowel procedures with MCC	Gastroenterology	39	$28,971	**$1.13M**

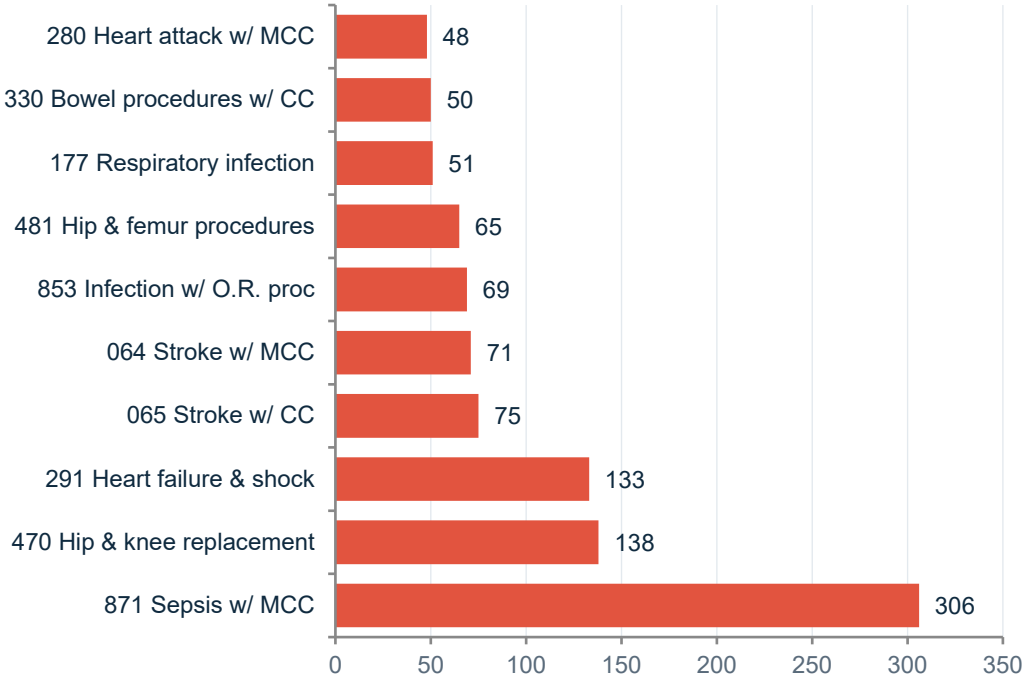
Concentration risk. The top eight DRGs deliver $17.3M — roughly a third of all Erlanger inpatient Medicare payments. Sepsis alone is the largest line and is medical, not surgical, volume.

High-value, low-volume anchors. ECMO and craniotomy cases average $33k–$137k per discharge. Twenty ECMO cases generate more revenue than 138 joint replacements.

Source: CMS Medicare Inpatient Hospitals by Provider and Service, 2024. Payments estimated from reported averages; DRGs with fewer than 11 discharges are suppressed by CMS and excluded.

Highest-volume inpatient DRGs and what each case is worth

Erlanger, 2024 — discharge count against average Medicare payment per discharge



Volume is medical, value is surgical

Sepsis, heart failure, stroke and respiratory infection make up over half the top-ten volume at $8,000–$14,500 per case. The surgical DRGs are two to four times richer per discharge but far smaller in count.

Implications for growth

- Medical volume responds to referral capture and ED throughput, not service-line marketing.
- Joint replacement is the one elective, schedulable line at scale — and the one Erlanger already leads.
- Stroke splits across two DRGs; combined, 146 cases and $1.6M make neurology a top-three line.

Source: CMS Medicare Inpatient Hospitals by Provider and Service, 2024. Original Medicare fee-for-service discharges only.

Outpatient: a quarter of the Medicare book, and it is holding

Erlanger outpatient APCs — 4,325 services and $17.9M in Medicare payments, 2024

APC	Ambulatory payment classification	Svcs	Avg pmt	Est. pmt
5115	Level 5 musculoskeletal procedures	359	$9,399	**$3.37M**
5213	Level 3 electrophysiologic procedures	73	$17,907	**$1.31M**
5114	Level 4 musculoskeletal procedures	274	$4,657	**$1.28M**
5362	Level 2 laparoscopy and related services	142	$6,823	**$0.97M**
5375	Level 5 urology and related services	240	$3,435	**$0.82M**
8011	Comprehensive observation services	366	$1,798	**$0.66M**
5361	Level 1 laparoscopy and related services	155	$3,780	**$0.59M**
5232	Level 2 ICD and similar procedures	21	$26,414	**$0.55M**

Outpatient held while inpatient fell

Erlanger outpatient payments moved $17.0M → $17.9M → $17.9M from 2022 to 2024 on essentially flat volume, while inpatient discharges dropped 13%. Outpatient is now 25% of the Medicare book.

Orthopedics leads here too

All reported musculoskeletal APCs deliver about 974 services and $5.64M. Levels 4 and 5 alone account for 633 services and $4.65M.

Market context. Across the three hospitals, 2024 outpatient Medicare payments run $26.9M at Memorial, $17.9M at Erlanger and $6.9M at Parkridge. Erlanger holds 34.5% of outpatient payments against 29.2% of inpatient discharges — the outpatient position is the stronger of the two.

Cardiac work has shifted setting

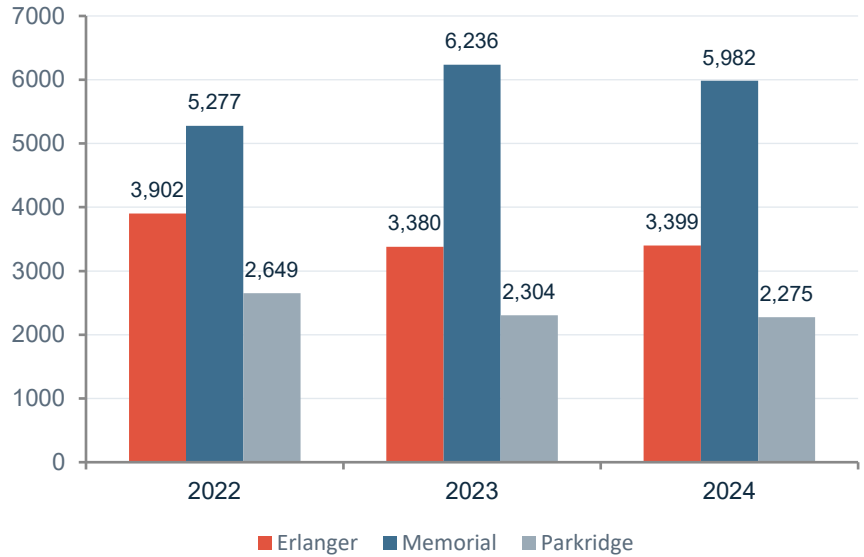
Electrophysiology and ICD implants add $1.86M outpatient — evidence that part of the inpatient cardiology decline is migration, not pure loss.

Source: CMS Medicare Outpatient Hospitals by Provider and Service (APC level), 2022–2024; low-volume cells suppressed. Beneficiary counts are summed across APCs (4,027), so a patient using several APCs is counted more than once — not a unique-patient count.

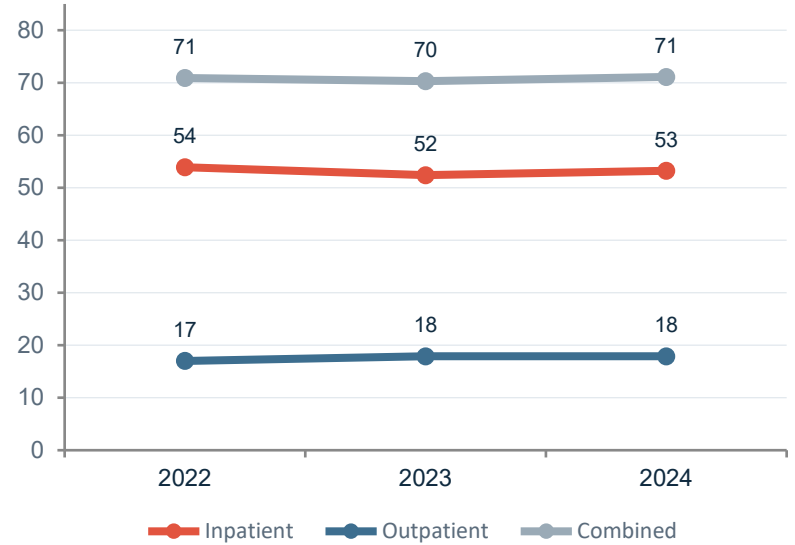
How volume and payments have moved, 2022–2024

Chattanooga acute-care market — three reporting hospitals, Original Medicare fee-for-service

Inpatient discharges



Erlanger Medicare payments ($M), inpatient vs outpatient

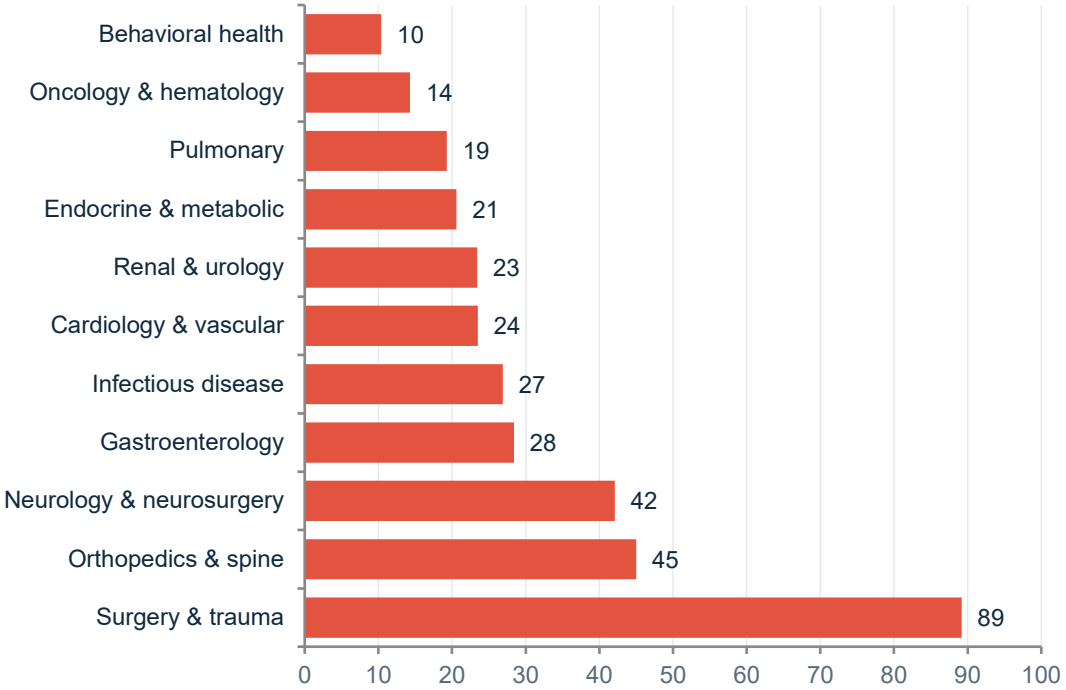


Read this together. Erlanger's inpatient discharge share fell from 33.0% to 29.2%, yet total Medicare payments were flat at about $71M — revenue per case rose and outpatient absorbed the shift. Memorial took the inpatient volume; Parkridge is shrinking on every measure.

Source: CMS Medicare Inpatient and Outpatient Hospitals by Provider and Service, 2022–2024. Erlanger reported as Erlanger Medical Center through 2023 and Erlanger Health System in 2024, same CCN 440104.

Inpatient market position by service line, 2024

Erlanger's share of Medicare discharges across the three reporting Chattanooga hospitals



Defend

Trauma, orthopedics and neurosurgery are the franchise. Erlanger takes nine of every ten trauma discharges and 45% of joint replacement.

Recapture

Cardiology at 23.5% is the largest addressable inpatient gap: Memorial books 1,340 cardiac discharges to Erlanger's 559.

Build or partner

Oncology (14.3%) and behavioral health (10.4%) are near-absent from Erlanger's inpatient Medicare book.

Source: CMS Medicare Inpatient Hospitals by Provider and Service, 2024; service lines assigned by the data package's keyword rule (first matching rule wins). Market = Erlanger, Memorial and Parkridge only.

02

Quality and experience

How Erlanger stands against Memorial and Parkridge on clinical measures and on what patients report

Quality standing against Chattanooga peers

CMS Hospital Compare measure groups — counts of measures better than, no different from, or worse than the national rate

	Erlanger Medical Center	Memorial Healthcare System	Parkridge Medical Center
CMS overall star rating	**4 of 5**	5 of 5	4 of 5
Ownership	**Government — hospital district**	Non-profit — church affiliated	Proprietary
Mortality (better / same / worse)	**0 / 8 / 0**	0 / 7 / 1	0 / 8 / 0
Safety of care (better / same / worse)	**3 / 5 / 0**	3 / 5 / 0	2 / 6 / 0
Readmission (better / same / worse)	**3 / 7 / 1**	2 / 9 / 0	0 / 6 / 3
Emergency services	**Yes**	No	Yes

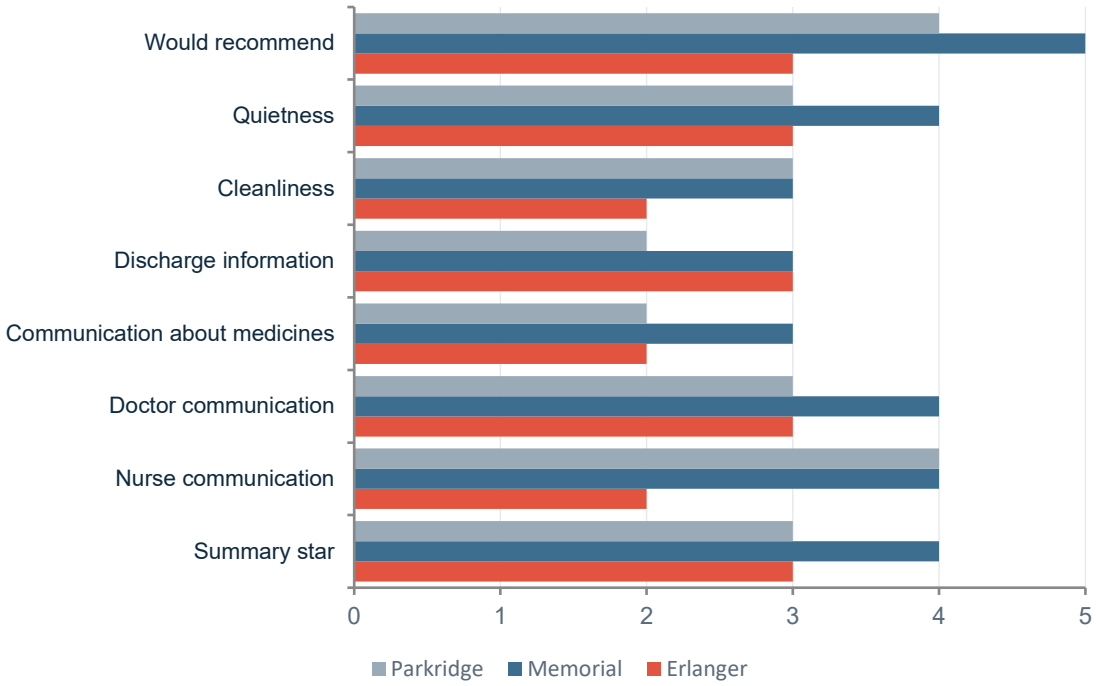
How to read it

Clinically, Erlanger is competitive: no mortality measure falls below the national rate, and it matches Memorial on safety while beating Parkridge on readmissions, where Parkridge is worse than national on three of nine reported measures. Against the one-star gap to Memorial on the overall rating, patient experience is the clearest measured gap — see the next slide.

Source: CMS Hospital General Information, current release as extracted 11 Aug 2026. Ratings reflect CMS reporting cycles and may lag current operations.

Patient experience is the clearest competitive gap

HCAHPS star ratings, survey period 1 Jul 2024 – 30 Jun 2025



67%

of Erlanger patients rate the hospital 9 or 10

Memorial 76% · Parkridge 69%

18%

survey response rate, 776 completed surveys

Memorial 23% · Parkridge 16%

2 stars

nurse communication and cleanliness

The two lowest scores in the market

Linear scores confirm it: Erlanger 89 on nurse communication and 74 on medication counselling, below Memorial on every composite.

Source: CMS HCAHPS — Hospital, survey period 1 Jul 2024 to 30 Jun 2025. Survey-based; compare response counts and periods before drawing firm conclusions.

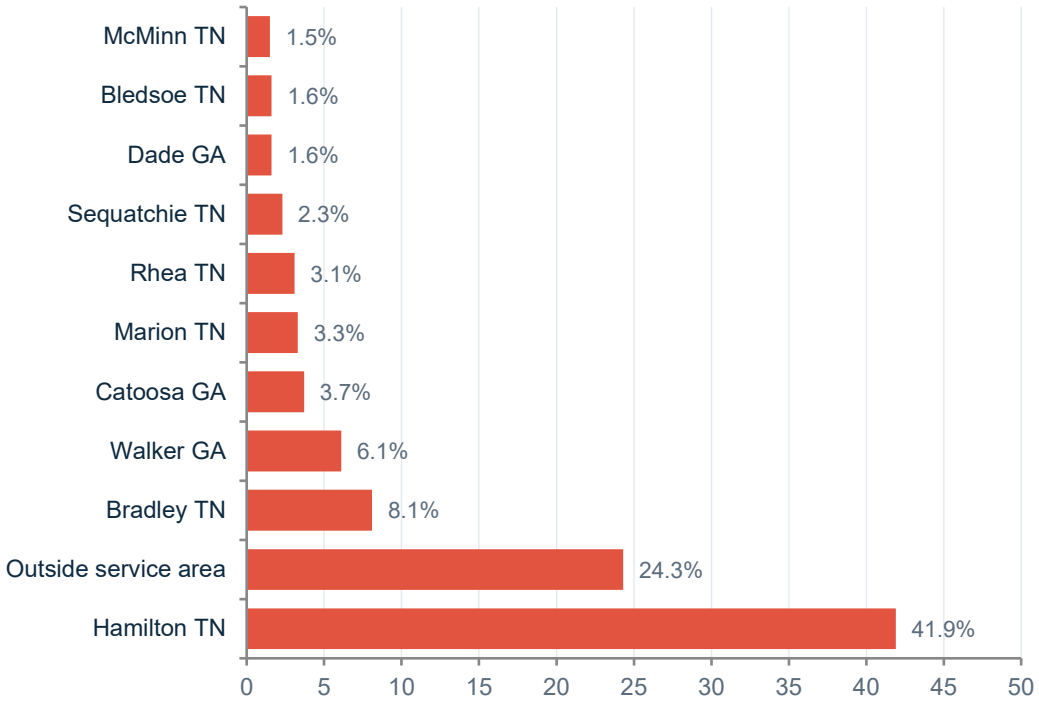
03

Who the community is

Where Erlanger's patients come from, where disease burden concentrates, and which service lines the need points to

Where Erlanger's patients actually come from

Main-campus discharge origin by county, FY 2024 — 25,006 discharges, adult and children's hospitals combined



The base is narrower than the map suggests

Hamilton County alone supplies 41.9% of main-campus discharges. The twelve secondary counties together contribute 33.8%, and Bradley, Walker and Catoosa are more than half of that.

A quarter of volume comes from beyond the service area

6,069 discharges — 24.3% — originate outside the thirteen defined counties. That is referral and transfer volume, and it rests on relationships rather than local market share.

Erlanger East is a local asset, not a regional one

60.4% of East's reported discharges come from Hamilton and only 17.1% from outside the service area — a different catchment from the main campus, and a different growth lever.

Source: Erlanger 2025 Community Health Needs Assessment, Section D, FY 2024 patient-origin table. Adult (20,690) and Children's Hospital (4,316) discharges combined; all payers, not Medicare only. Blank Erlanger East cells mean not separately reported and are not treated as zero.

County view: high disease burden, thin Erlanger presence

Chronic-disease burden index vs. share of Erlanger main-campus discharges, FY 2024

County	Population	Burden index	Uninsured adults	No transport to care	Erlanger share of discharges
Sequatchie, TN	16,909	**23.1**	11.4%	9.0%	2.3%
Bledsoe, TN	14,798	**23.0**	12.5%	11.5%	1.6%
Rhea, TN	33,730	**22.6**	12.1%	9.4%	3.1%
Grundy, TN	13,783	**22.0**	10.6%	8.4%	1.1%
Marion, TN	29,094	21.9	8.8%	9.0%	3.3%
Walker, GA	68,915	**20.3**	13.8%	8.2%	6.1%
Bradley, TN	110,616	20.1	10.3%	8.3%	8.1%
Hamilton, TN	374,682	18.6	8.5%	7.0%	41.9%

The rural rim is a higher-need area. The four highest-burden counties hold about 79,000 people and roughly 8% of Erlanger's discharges. COPD runs to 14.3% and coronary heart disease to 10.4% there, against 7.9% and 7.1% in Hamilton.

But county averages hide more than they show. Hamilton looks healthiest of all at 18.6 — and contains the single worst ZIP code in the service area. The next slide breaks it open.

Sources: CDC PLACES 2024 county release (burden index = mean of coronary heart disease, diabetes, COPD, stroke, high blood pressure, obesity and depression); Erlanger 2025 CHNA Section D for FY 2024 discharge shares.

ZCTA view: the sharpest access gap is in the urban core

63 service-area ZIP areas (ZCTAs) ranked on disease burden and access barriers — burden ranges from 14.7 to 26.9

ZCTA	County	Population	Burden index	Uninsured	No vehicle	Poverty	Median income
37410	Hamilton, TN	4,129	**26.9**	21.5%	32.0%	43.4%	$28,708
37381	Rhea, TN	10,711	**24.0**	12.7%	5.8%	18.8%	$56,869
37406	Hamilton, TN	13,364	**23.3**	15.3%	18.3%	30.4%	$44,850
37380	Marion, TN	6,688	**23.3**	13.2%	5.8%	27.0%	$59,304
37338	Bledsoe, TN	3,309	**23.0**	14.1%	2.0%	24.5%	$49,592
37407	Hamilton, TN	9,156	**22.9**	31.0%	14.6%	45.4%	$35,845
37402	Hamilton, TN	3,953	**22.7**	13.9%	37.2%	36.5%	$24,214
37404	Hamilton, TN	14,737	**22.5**	20.3%	8.6%	23.8%	$56,637
37350	Hamilton, TN	2,111	**15.1**	4.0%	1.6%	2.3%	$201,000

Five urban ZCTAs, 45,000 residents. ZCTAs 37402, 37404, 37406, 37407 and 37410 are the highest-burden areas in Hamilton County. Poverty runs 24–45%, uninsured to 31%, and up to 37% of households have no vehicle at all.

A 12-point burden gap inside one county. Hamilton contains both the highest-burden ZCTA in the service area (37410 at 26.9) and the lowest (37350 at 15.1). Transport is a likely access barrier in the urban core.

Sources: CDC PLACES 2024 ZCTA release (model-based crude prevalence; burden index as defined on the previous slide); U.S. Census ACS 2020–2024 5-year estimates for vehicle access, poverty and income. ZCTAs under 2,000 residents excluded. ZCTAs are Census tabulation areas, not postal ZIP codes.

Which of the five service lines shows the strongest demand

Inpatient and outpatient Medicare position (2024) read against community disease prevalence

STRONGEST

Orthopedics

598 IP discharges · 45.0% share
$10.1M IP + $5.6M OP

The only major line growing — up 17.7% on 2023 — and the largest outpatient APC group. Arthritis affects 31–38% of adults across the service area.

SCALE IT

LARGEST GAP

Cardiology

559 IP discharges · 23.5% share
$9.7M IP + $1.9M OP

Inpatient volume down 28.7% since 2022 against Memorial's 1,340 cases, but outpatient EP and ICD work is growing — part of the loss is migration.

RECAPTURE

UNMET NEED

Behavioral health

32 IP discharges · 10.4% share
Rates vary about 5x across payers

Parkridge books 214 of the market's 307 cases. Depression runs 29–32% region-wide and reaches 31.8% in Sequatchie.

BUILD OR PARTNER

SUB-SCALE

Oncology

58 IP discharges · 14.3% share
Widest price spread: about 5x

Memorial takes 310 cases. Cancer prevalence is 8.6% in Hamilton and above 10% in Marion and Meigs; colorectal screening lags at 61.5% in the rim.

ASSESS

STILL UNMEASURED

Women's health

21 IP discharges · $0.5M
Price data now available

Medicare fee-for-service captures almost no obstetric volume by design. Negotiated rates exist, but volume needs all-payer or state discharge data.

OBTAIN DATA

Verdict: orthopedics carries the strongest proven demand across both settings; cardiology carries the largest recoverable inpatient volume; behavioral health carries the widest gap between community need and Erlanger capacity.

Sources: CMS Medicare Inpatient and Outpatient Hospitals 2022–2024; CDC PLACES 2024; Erlanger price transparency files. Medicare fee-for-service under-represents obstetrics, paediatrics and most commercially insured elective care.

04

What the market pays

Negotiated outpatient facility rates across 44 payers, and the filters that make the spread figures defensible

Where negotiated prices differ most across insurers

Erlanger Chattanooga hospitals — average negotiated dollar rate by payer, outpatient facility rates as of 30 Sep 2025

Service line	BCBST	UHC	Cigna	Aetna	Humana	CareSource	Spread
Oncology & hematology	$4,246	$1,771	$2,771	$2,159	$2,047	$10,613	**5x**
Behavioral health	$1,705	$637	$1,048	$658	$790	$4,605	**5x**
Imaging & radiology	$821	$365	$388	$1,000	$223	$1,096	**28x**
Emergency & urgent care	$5,033	$2,970	$3,374	$2,170	$2,704	$11,143	**26x**
Cardiology & vascular	$5,814	$3,299	$3,545	$2,878	$2,603	$13,885	**24x**
Orthopedics & spine	$1,780	$1,477	$1,615	$1,447	$1,139	$4,502	**9x**

Widest variation is in the lines Erlanger is smallest in

Oncology and behavioral health — the two service lines with the weakest inpatient share — also carry the widest price dispersion under this view, each spanning roughly 5-fold across 44 payers. Thin volume means thin negotiating leverage.

Among the big five, BCBST leads and Humana trails

BCBST pays roughly twice Humana on cardiology ($5,814 vs $2,603) and more than twice UHC on oncology and behavioral health. Imaging inverts the pattern: Aetna pays $1,000 against Humana's $223.

Orthopedics is the disciplined contract

A 9x spread and the tightest band among the major payers ($1,139–$1,780) — the pricing profile of a line with real market share behind it.

Source: Erlanger hospital price transparency machine-readable files, as-of 30 Sep 2025, published 25 Jan 2026 — 6,835 aggregated records representing approximately 3.89M standardized rate observations for the Chattanooga file, across 44 payers. Figures are filtered to Chattanooga hospitals, outpatient setting, facility rates. Spread = highest ÷ lowest payer average among payers with 30+ rate observations. Rate methodologies differ by contract (fee schedule, case rate, percent of billed charges), so figures are indicative rather than strictly like-for-like.

How the price figures were calculated

Filters, exclusions and the spread definition behind the negotiated-rate slide

Step	Filter or rule applied	Why	Effect on the figures
01 Market	Chattanooga-area Erlanger hospitals	Keeps one market in view	Excludes other campuses
02 Setting	Outpatient only	Inpatient case rates are not per-service	Drops inpatient terms
03 Rate type	Facility rates only	Professional fees price differently	Drops professional lines
04 Reliability	Payers with 30+ observations	Stops a thin contract setting the range	Drops long-tail payers
05 Spread	Highest ÷ lowest payer average	A ratio the board can read	Oncology and behavioral health near 5x

Why the earlier 47x and 45x figures were wrong

Those spreads pooled inpatient with outpatient and professional with facility rates and included payers with very few observations. Applied consistently, the outpatient facility view puts both lines near 5x.

What the spread does and does not prove

A widespread shows dispersion in contracted terms, not that a payer is underpaying. Methodologies differ, and service lines are keyword-mapped from code descriptions, so read it as a prompt to inspect a contract.

Source: Erlanger hospital price transparency machine-readable files, as-of 30 Sep 2025, published 25 Jan 2026 — 6,835 aggregated records representing approximately 3.89M standardized rate observations for the Chattanooga file, across 44 payers. Rebuild in Tableau by applying steps 01–04 as workbook filters before any spread calculation.

05

From data to action

The Tableau workbook that keeps these views current, the model behind it, and the moves the evidence supports

Standing this up in Tableau: five dashboards

One workbook, five tabs — each answering one of the questions this deck opened with

01	**Service Line Performance** Inpatient DRG · Outpatient Volume	Discharges, payments and average payment by service line and setting, with year toggle and DRG/APC drill-down.	**Service-line leads · monthly**
02	**Market Share & Trend** Market Summary · Inpatient DRG	Erlanger vs. Memorial vs. Parkridge share by line and year, with a trend sparkline per service line.	**Executive · quarterly**
03	**Quality & Experience** Facility Quality · HCAHPS	Star ratings and HCAHPS composites benchmarked against both competitors, flagged where Erlanger trails.	**Quality & nursing · quarterly**
04	**Community Access Map** ZIP Access · Community Health · Patient Origin	Filled ZCTA map of burden index against vehicle access, uninsured rate and poverty, layered with patient origin.	**Community health · semi-annual**
05	**Payer Price Explorer** Negotiated Rates	Rate distribution by payer, service line and methodology, with the spread metric and outlier list.	**Contracting · on each file refresh**

Every view above is buildable from the Excel package's 12 tabs, including its nine analytical data tables — no additional feed required. Outpatient service-line classification must be created in Tableau, as the Outpatient Volume table has no service-line field. Owner and cadence are proposed, not agreed.

Dashboard 1 — Service Line Performance

Discharges, payments and average payment by service line and setting, with DRG and APC drill-down

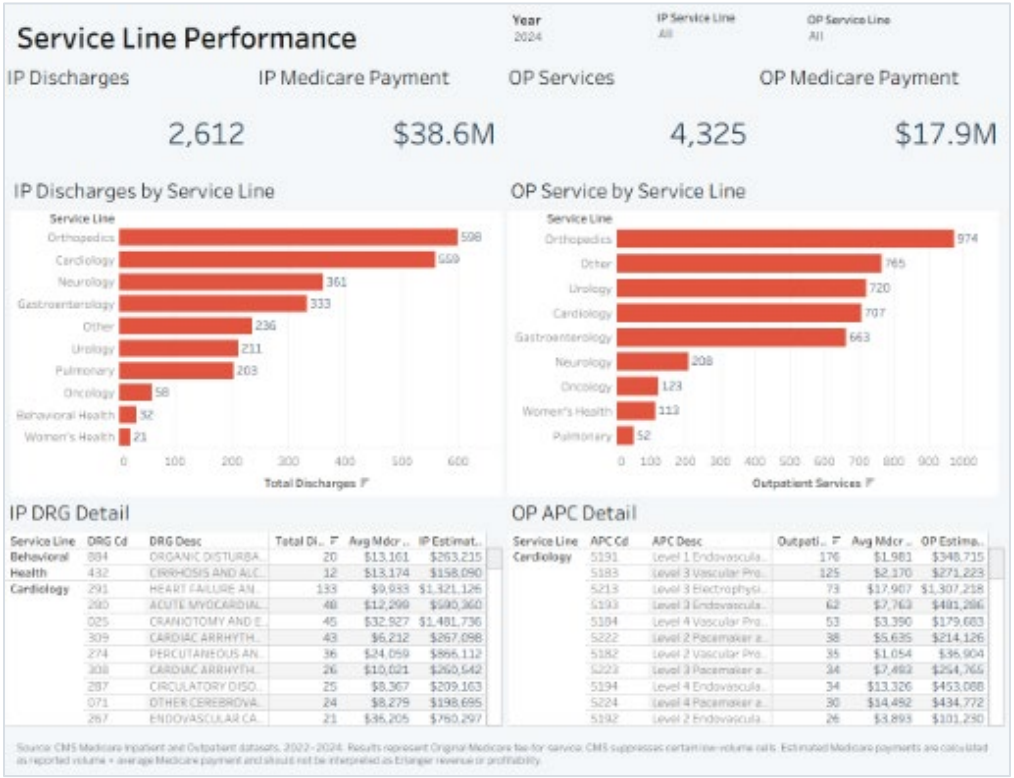


Tableau mock-up. 2,612 inpatient discharges / $38.6M and 4,325 outpatient services / $17.9M for 2024; orthopedics leads both settings.

Dashboard 2 — Market Share & Trend

Erlanger against Memorial and Parkridge by service line and year, with a per-line trend view

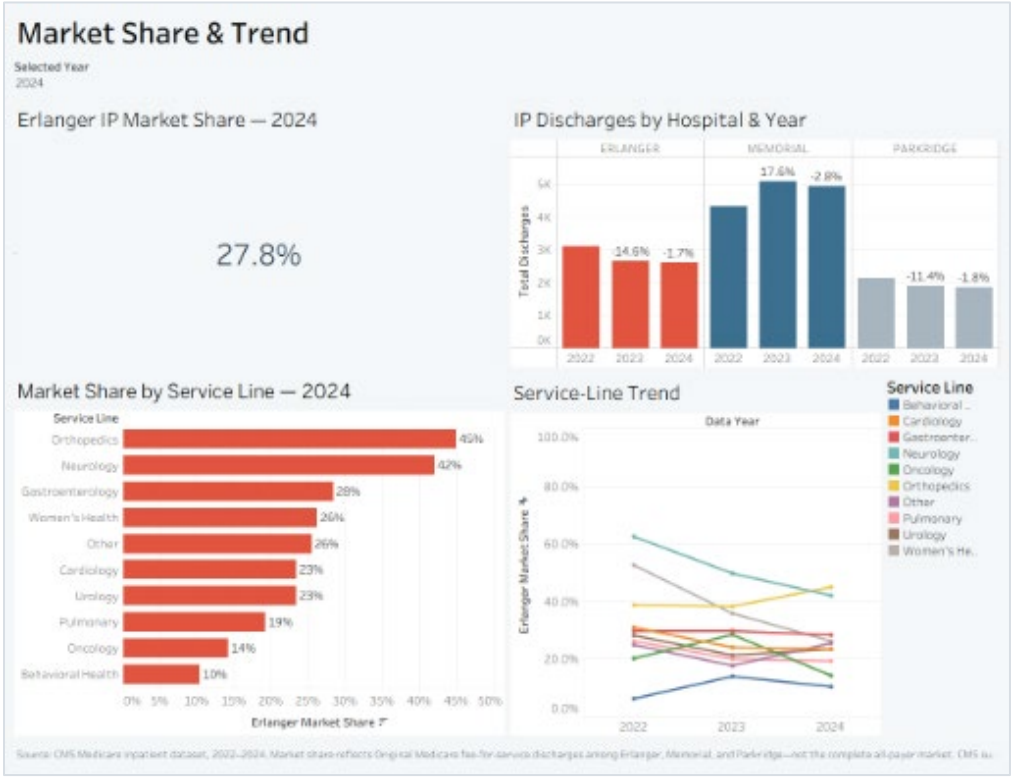


Tableau mock-up. Erlanger holds 27.8% inpatient Medicare market share in 2024; orthopedics 46% and behavioral health 10%.

Dashboard 3 — Quality & Experience

Star ratings and HCAHPS composites benchmarked against both competitors, flagged where Erlanger trails



Tableau mock-up. Overall CMS rating of 4 with an 18% survey response rate; HCAHPS bars show where Erlanger sits below peers.

Dashboard 4 — Community Access & Patient Origin

Filled ZCTA burden map read against vehicle access, uninsured and poverty rates, layered with patient origin

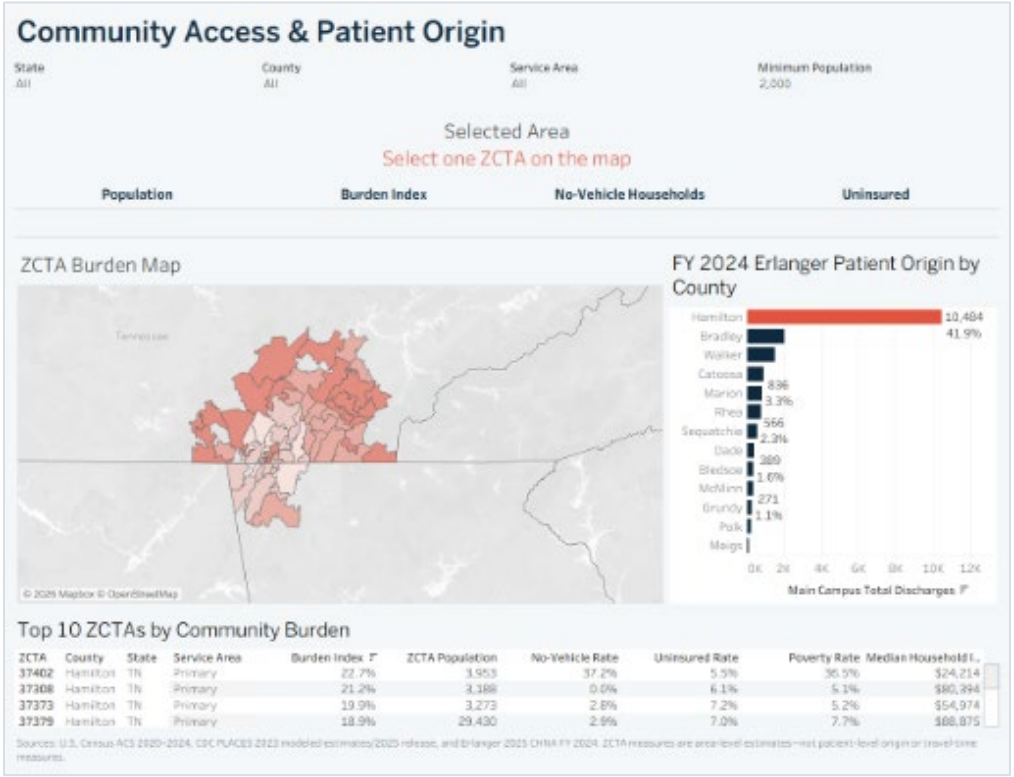


Tableau mock-up. Highest-burden ZCTAs are Hamilton County primary-service-area ZCTAs; Hamilton supplies 41.9% of main-campus discharges.

Dashboard 5 — Payer Price Explorer

Negotiated outpatient facility rate variation by payer, service line, plan and methodology

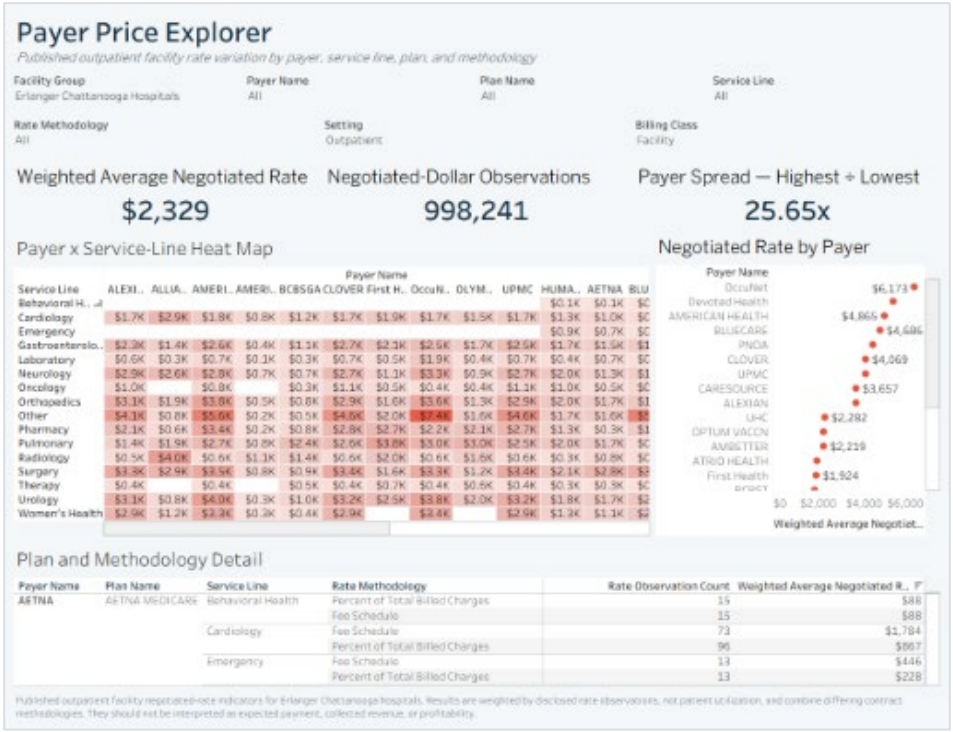


Tableau mock-up. Chattanooga outpatient facility view: $2,329 weighted average negotiated rate across 998,241 dollar observations. The 25.65x spread here is highest-to-lowest across all payers, unfiltered; the roughly 5x figure on the pricing slides is the same ratio within one service line, limited to payers with 30 or more observations. Different measures, not a conflict.

The data model behind it

A Tableau relationship model — not one large physical join, as the data package itself recommends

Primary table	Related table	Join key	What the relationship gives you
Inpatient DRG	Facility Quality	CCN ↔ facility_id	Hospital attributes and CMS star rating on every volume view
Outpatient Volume	Facility Quality	Rndrng_Prvdr_CCN ↔ facility_id	The same attributes, so the two settings compare cleanly
HCAHPS	Facility Quality	facility_id	Patient experience benchmarked in one filter context
Community Health	Patient Origin	county_fips	Need indicators read against where discharges originate
ZIP Access	Patient Origin	county_fips	ZIP-level need with county patient-origin context
ZIP Access	Community Health	county_fips	ZIP-to-county comparison without a physical join
Inpatient DRG	Service Line Rules	service_line	Auditable record of the keyword classification

Grain differs by table — do not blend

Inpatient is hospital × year × DRG, outpatient is hospital × year × APC, ZIP Access is ZCTA × metric × estimate type. Relationships preserve those grains; a physical join will double-count payments.

Refresh cadence follows the source, not the calendar

CMS utilization files update annually, quality and HCAHPS quarterly, PLACES annually, price transparency monthly. Stamp each view with its own as-of date rather than one workbook date.

Source: data package Project Guide, recommended and additional Tableau relationships. The Excel package holds 12 tabs, including nine analytical data tables; the Outpatient Volume table carries no service-line field, so that classification must be built in Tableau. Row counts: Inpatient DRG 1,048 · Outpatient Volume 504 · HCAHPS 204 · Community Health 520 · ZIP Access 3,338 · Negotiated Rates 12,772.

What the moves could be worth

Illustrative annual Medicare impact, sized from 2024 average payment per discharge

Move	Service line	Basis for the estimate	Annual Medicare impact	Confidence
Recapture cardiology referrals	Cardiology & vascular	10-point inpatient share shift, about 78 of the 780 discharges now with Memorial	**~$4.0M**	Medium
Sustain orthopedic growth	Orthopedics & spine	Holding the 2023-24 growth rate on the $15.7M combined inpatient and outpatient base	**~$1.5-2.0M**	Medium-high
Build behavioral health capacity	Behavioral health	Moving from 10.4% to 25% of the market's 307 discharges, about 45 additional cases	**~$0.6M**	Low - needs case-rate detail
Reprice the outlier contracts	Oncology · behavioral health	Lifting the lowest-paying commercial terms toward the payer median	**Not sizable here**	Commercial volume data required
Fix patient experience	Nurse communication · cleanliness	Value-based purchasing and referral effects rather than direct volume	**Indirect**	Not estimable from this package

Together the two sizable moves are worth roughly $5.5-6.0M a year in Medicare payments — before the cost to deliver them.

Illustrative only. Estimates apply 2024 average Medicare payment per discharge to a stated share shift. Gross payments, not margin; commercial and Medicare Advantage volume is not in this package.

Where to act first

Five moves ordered by the size of the gap the data now evidences

01	**Recapture cardiology referrals** Cardiology & vascular	**559 vs Memorial's 1,340 discharges**	Roughly 780 Medicare discharges sit with a single competitor in a line Erlanger already staffs. A 10-point share shift is worth about $4M in inpatient payments, and outpatient EP shows the demand is there.
02	**Protect and extend the orthopedic franchise** Orthopedics & spine	**45% IP share · $5.6M outpatient APCs**	The only line growing in both settings, and the tightest payer contract at a 9x spread. Capacity, rather than demand, is the likely constraint to test.
03	**Fix nurse communication and cleanliness** Patient experience	**2 of 5 stars on both, lowest in market**	These two composites carry the one-star overall gap to Memorial. They are operational, not clinical, and move within a single survey cycle.
04	**Open access in the urban core, not only the rim** ZCTAs 37402 · 37404 · 37406 · 37407 · 37410	**45,000 residents, up to 37% with no vehicle**	The highest-burden ZCTA in the whole service area sits in the urban core near the main campus. Transport support and neighbourhood clinics are a concentrated opportunity alongside rural outreach.
05	**Reprice the outlier contracts** Oncology · behavioral health · imaging	**Roughly 5x rate spreads across payers**	The widest dispersion sits in the lines with the least volume behind them. Build the volume case first, then reopen the lowest-paying commercial terms.

Payment estimates derived from 2024 average Medicare payment per discharge. Remaining limits: Medicare files are not all-payer and suppress low-volume cells; PLACES values are model-based estimates; negotiated rates are indicative given differing contract methodologies.

A 90-day sequence

What gets built, decided and measured — and in what order

DAYS 0–30

Stand up the workbook

- Build the five Tableau tabs, adding an outpatient service-line classification in Tableau
- Publish to server with row-level access for service-line leads
- Validate service-line keyword rules with clinical leadership

Analytics + service-line leads

DAYS 31–60

Test the big theses

- Referral audit on the 780-discharge cardiology gap
- Nurse-communication and cleanliness improvement plan in place
- Capacity model for orthopedics across both settings

Operations + nursing

DAYS 61–90

Commit the cases

- Urban-access proposal for the five highest-burden ZCTAs
- Reopen the widest-spread payer contracts with a volume case
- Board-ready business cases with a measurement baseline

Strategy + contracting

Measure of success at day 90: a single published workbook that every service-line review runs from, and three funded business cases with a baseline already captured.

Sequence assumes no new data acquisition; it runs on the 12-tab Excel package and its nine analytical data tables. All-payer or state discharge data would be required to extend the model to women's health and commercially insured elective volume.

How to read this — and where it stops

The limits that must travel with every dashboard view

Medicare is not the market

CMS inpatient and outpatient files cover Original Medicare fee-for-service only. Medicare Advantage, commercial, Medicaid and self-pay volume are absent — so obstetrics, paediatrics and most elective commercial work are under-represented by design.

Suppressed cells understate totals

CMS suppresses low-volume DRG and APC cells, so the discharge and service counts here are not full hospital census totals. Treat them as comparable across hospitals, not as absolute activity.

Charges are not prices or costs

Submitted charges are neither costs, negotiated prices, collected revenue nor profit. Negotiated rates aggregate 8.46 million published rows across differing methodologies — fee schedule, case rate, per diem, percent of billed charges — so they are indicative, not like-for-like.

PLACES estimates are modelled, not counted

CDC PLACES values are model-based community estimates and do not measure Erlanger's patients. ZCTAs are Census statistical areas, not postal ZIP codes.

Service lines are an analytic construct

Service-line assignment is a keyword classification documented in the Service Line Rules sheet — not an official CMS grouping. Validate it with clinical leadership before it drives an investment decision.

Facility scope varies by file

Erlanger's Chattanooga campuses report together under CCN 440104. Memorial Hixson (CCN 440103) has no current CMS quality row and is excluded. The consolidated price file does not identify which individual Erlanger hospital a rate applies to.

All figures in this deck derive from public sources extracted 11 Aug 2026: CMS Medicare Inpatient and Outpatient Hospitals by Provider and Service 2022–2024; CMS Hospital General Information and HCAHPS; CDC PLACES 2024; U.S. Census ACS 2020–2024; Erlanger 2025 CHNA; hospital price transparency files as of 30 Sep 2025.